BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show sequence, not equal lengths. Count 12 weeks from first HER2 treatment (after AC where applicable): around weekly antibody dose 13 or q21-day cycle 5. Preserve elapsed time across later phase / route changes and delays. The treating team owns booking, result review and action when a test is overdue.

Check	Before starting	Chemotherapy visits	Phase change / 12 weeks	Continuation / local care	Completion / follow-up
Blood tests	Baseline FBC, renal and liver profile	FBC, renal and liver profile before each cycle. Weekly-day checks follow the active regimen and local delivery policy.	Reassess before a new phase	Companion drugs / abnormal results determine additional tests	Follow persistent abnormalities and the next plan
Echo / cardiac assessment	Baseline before AC; reassess before trastuzumab starts.	Earlier assessment for symptoms or cardiac concern	Count 12 weeks from first HER2 treatment (after AC where applicable): around weekly antibody dose 13 or q21-day cycle 5. Preserve elapsed time across later phase / route changes and delays.	Every 12 weeks during HER2 treatment, including continuation.	EMA: cardiac checks every 6 months after trastuzumab ends until 24 months; after anthracyclines in early disease, yearly to 5 years from the last trastuzumab dose or longer if function falls.
ECG / rhythm	Baseline ECG in the AC source; repeat for symptoms / cardiac indication. Echo is a separate test.	Symptoms, interacting medicines or clinical indication	An ECG does not replace an echo	Repeat if symptoms or the cardiac plan require it	Agree follow-up if abnormal
Imaging / markers	Document baseline disease and purpose of scans	Review symptoms and response	No routine CT solely because chemotherapy ends	Response imaging guides surgery in neoadjuvant care. Otherwise image for symptoms or a clinical indication.	Annual mammography of remaining breast tissue. No routine CT / PET or tumour markers for asymptomatic distant-recurrence screening.
Bone health / DEXA	Assess endocrine treatment and fracture risk	Not a routine scan at each cycle	Baseline DEXA when indicated	DEXA before aromatase inhibitor / ovarian suppression when indicated; repeat according to fracture risk, results and endocrine plan.	Risk- and result-based follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00316 v6 · official source · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Herceptin product information

THE WHOLE TREATMENT JOURNEY

Dose-dense AC → fortnightly paclitaxel + weekly trastuzumab: the whole pathway

HER2-positive neoadjuvant or adjuvant high-risk breast cancer

Four AC cycles every 14 days, then four fortnightly paclitaxel treatments with trastuzumab weekly on Days 1 and 8 of each 14-day block (eight weekly antibody doses). Trastuzumab then continues every 21 days to complete one year total; it is not given with AC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before systemic treatment	Chemotherapy phase	Next phase / reassessment	Local / continuing treatment	Follow-up / change
Cancer medicines NCCP 00316	Confirm indication, organ function and access	AC ×4	Taxane with trastuzumab	HER2 continuation under the agreed plan	Review outcome and next treatment
Surgery EARLY	Choose preoperative or postoperative pathway	Neoadjuvant: systemic treatment before surgery	Neoadjuvant: surgery / pathology after recovery	Adjuvant: surgery already completed before systemic treatment	Follow the selected local-care pathway
Radiotherapy EARLY	Assess indication	Usually after chemotherapy in adjuvant care	Coordinate surgery and recovery	May overlap HER2-only treatment when agreed	Assess late effects / local follow-up
Blood tests NCCP 00316	Baseline FBC, renal, liver	FBC, renal and liver profile before each cycle. Weekly-day checks follow the active regimen and local delivery policy.	Review before transition	Continue according to active medicines	Follow abnormalities
Heart tests NCCP 00316 · SMPC · ESC	Baseline echo / MUGA	Before HER2 starts after AC	At 12 weeks from HER2 start	Every 12 weeks / 3 months during HER2	Agree post-treatment cardiac plan
Scans / markers EARLY · METASTATIC	Baseline staging as indicated	Clinical response / symptoms	Not automatic CT screening	Breast surveillance plan	Symptoms can trigger earlier tests
Endocrine / bone health EARLY · BONE	Check receptors / endocrine indication	Discuss the later plan	Start / continue when agreed	May overlap HER2 / RT if appropriate	Longer-term bone health and endocrine review

What each check is for

1 **Blood checks**

FBC, renal and liver profile before each cycle. Weekly-day checks follow the active regimen and local delivery policy.

Results help the team judge treatment safety.

Ask / act: Confirm the next test and who reviews it.

Sources: NCCP 00316

2 **Heart assessment**

Baseline before AC; reassess before starting trastuzumab. Then every 12 weeks during HER2 treatment.

Some heart changes occur without symptoms.

Ask / act: Keep the appointment and report new symptoms between scans.

Sources: NCCP 00316 · SMPC · ESC

3 **Imaging and markers**

Early breast follow-up uses planned breast imaging. Routine CT / PET, blood tumour markers or other distant-recurrence screening are not recommended without symptoms or a clinical indication. After local treatment: annual mammography of remaining breast tissue; additional imaging when indicated.

Each scan should answer a defined clinical question.

Ask / act: Ask when the result will be reviewed.

Sources: EARLY · METASTATIC

4 **Booking and handover**

At initiation, phase changes and completion

Expected tests can be missed without an owner.

Ask / act: Record booking owner, result reviewer, due window and escalation contact.

Sources: NCCP 00316

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-17 · Source reconciled; independent clinical, pharmacy and patient review pending · SACTCheck v0.84.0